



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : سعاد جمال عطيع ابراهيم
المحافظة : المنيا
الحى / المدينة : مغاغة
رقم البطاقة : 27304262401104 رقم التليفون : 01272200121
العنوان : قرية طنبدى
العمر (تاريخ الميلاد) : 26-04-1973 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 28-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام اشعه مقطعيه علي البطن والصدر والحوض
بالصبغة [11127]
التعليق على التشخيص : -----

قسم العلاج :
الأورام

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.
تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /----- /د. /----- /د. /----- /د. /-----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: سعاد جمال عطيح ابراهيم National ID: 27304262401104	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Contrast-enhanced CT Chest & abdomen and pelvis
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٧٣

٢٧٣ / ٢٧٣

فتح صلف
١٩/٨/٢٠٢٥

أحمد خال
مبطل

٢٧٣ / ٢٧٣

MDT FORM 14 / 9 / 20 25 G

Name	سار جمال عيسى	MRN	2354/2025
		ID	
Age	52 yr	Phone	0127 2200121
Menopausal status	Post	Marital status	
NRS SCORE		Family history	we
BMI		REASON OF MDT	27304262401104
CO morbidity	free		

Clinical Data

BX:

R

L

ER: 618

PR: 418

HER 2: 0C-yes

Ki67: 20%

HFL: 818

318

+3

15%

Sonomammography:

2020 → left MRM
→ Adj Tamoxifen for 5 ys

Mets Workup:

7/2025 - MRI Abd/Revis

+ left soft tissue → multiple
+ left Renal chaz HFL 2.1x2.2
+ left supraRenal mass 3.0x3.4x4.7

TNM

BX → ER 818
PR 618
HER2 +3
Ki67 15%

Recommendations:

Echo 66%.

CT CAP → enlarged liver w/ multiple HFL, largest 2.9x2.7 c
30/8/25 → left adrenal gland replaced by soft tissue mass 4.8x4.1 c
inseparable from tail of pancreas

MDT OUTCOME & PLAN

In total weekly + herceptin-IV

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name: سعاد جمال عطيه			
MR. : 881			
Detailed Medical Report			
			السيد
تحية طيبة وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
Postmenopausal metastatic breast cancer to liver, ER8/8 PR3/8 HER2 3+ with HFL biopsy left mammary right 2020. The patient has been on tamoxifen for 5 years until 8/2025, when liver metastases were noted on CT 12/2025. A radiation dose plan was prepared for week 15 of taxol 110 mg and cycle 5 of Herceptin; the patient was on Herceptin 16/12/2025. Metastatic breast cancer to liver and adrenal, HER2-enriched, currently receiving taxol and Herceptin IV. The patient is on week 19 of taxol 110 mg and cycle 7 of Herceptin 333 mg, with week 20 and week 21 doses at pharmacy. Weight 58 kg, height 154 cm. CT 7/12/2025 scheduled for week 22 of taxol 110 mg and cycle 8 of Herceptin 350 mg. Weeks 23 and 24 doses are at pharmacy. Metastatic breast cancer to liver on taxol and Herceptin. The last CT (pending) is for cycle 9 of Herceptin and week 25 of taxol, with weeks 26 and 27 at pharmacy. (3/2026 EF 64%). Day-care pharmacy visit for week 27 of taxol 110 mg. Labs were accepted on 11/3. Metastatic breast cancer to liver and suprarenal with J-catheter. A 3/2026 CT was reviewed for HFL + SD of rest. A 3/2026 echo showed EF 64%. Today the patient is on week 28 and cycle 10 of Herceptin. MDT request pending. Today the patient received cycle 11 of Herceptin IV and will start Femara for the next two cycles. Pharmacy noted weight 59 kg and administered cycle 11 Herceptin 355 mg with Femara. Echo 4/3 showed EF 64%. Visit on 3/6/2026 for cycle 13 of Herceptin and supportive therapy. CT 24/3/2026 showed bilobar HFL largest 9 × 10 mm. New CTs for cycle 14 of Herceptin 360 mg (weight 60) and Femara were planned. Today the patient received cycle 14 of Herceptin 360 mg IV, Femara, and supportive therapy. Echo pending. CT CAP was done without comparison; comparison pending. Anesthesia note: The patient with metastatic breast cancer had tramadol dose decreased to 50 mg per day but pain was not controlled; tramadol will be increased to 100 mg per day with cetral 500 mg every 8 hours.			
تحريراً في ,			
		التاريخ :	20-09-2026
		الوقت :	
		الطبيب المعالج :	dina makram
يعتمد ,			

Internal medicine note: The patient received cycle 15 of Herceptin IV, Femara, and supportive therapy. CT with comparison showed stable disease.

Medical oncology note: CT on 19/7/2026 showed stable disease of bilobar HFL largest 10 × 7 mm, EF 65%. The patient received cycle 16 of Herceptin 380 mg (weight 63) and Femara. The patient has completed 16 cycles of Herceptin.

Patient's Name: Souad Gamal Atiya
Date: 2026-07-04
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

Known case of breast carcinoma underwent left MRM. No available previous DICOM images for comparison.

(I) Chest:

- Right breast small lesion measures 14 x 10 mm for sonomammography.
- Bilateral lower lobar mosaic attenuation of small airway disease.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- The liver is enlarged in size showing homogenous CT density with bilobar hepatic hypodense focal lesions, the largest seen at segment VI measures 10 x 7 mm.
- Patent and homogeneously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Left kidney shows moderate backpressure changes. Soft tissue sheet is seen encasing the left pelvi-ureteric junction measuring 25 x 22 mm.
- Bulky left suprarenal gland, no sizable masses are noted .
- Normal CT appearance of the spleen, pancreas, right adrenal gland, right kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses .
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.

OPINION:

- Multiple bilobar hepatic focal lesions for dynamic MR study.
- Moderate left hydronephrosis with soft tissue sheet encasing the left pelvi-ureteric junction ? scar tissue ? other for PET-CT assessment.

Dr.sherif Wagdy

Dr.sherif Wagdy

Patient Name : الأمتانه / معاد جمال عطيه ابراهيم	Request No. : 13003306
Patient No. : 27304262401104	Request Date : 19/08/2025
Path No. : SR-1141/25	Verification Date : 21/08/2025
Sex : Female	Financial Category : جوماتاف روسى - نفقة الدولة
Age : 52 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

CLINICAL DATA

Hepatic focal lesion , history of cancer breast .

NATURE OF SPECIMEN

Block & slide revision .

GROSS EXAMINATION

Two blocks labelled by (977426) & 7 slides were received.

MICROSCOPIC EXAMINATION

- * Histological type: Metastatic adenocarcinoma
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade 2
- * IPT (Positive control showed appropriate reactivity) :
- * Gata3 : Positive .
- * CK7: Strong diffuse .
- * ER: Strong nuclear reaction in 80 % tumor cells (Allred score 8/8).
- * PR: Weak nuclear reaction in 10 % tumor cells (Allred score 3/8).
- * HER2: Strong complete membrane reaction in >10 % of tumor cells (score 3+).
- * Ki 67: 15 %

DIAGNOSIS

Hepatic focal lesion , block & slide revision :
Metastaic adenoxcarcinoma of breast origin .

IPT:

- * ER: Positive (score 8/8).
- * PR: Positive (score 3/8).
- * HER2: Positive (score 3+).

Verified by :
Dr.Heba Hasan



Patient Name: Souad Gamal Atteya Ibrahim	Date : 07/12/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 52 Year(s)
Medical ID: 2354/2025	Pat. ID: 111192

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

Clinical History:

- Known case of left breast cancer, copared to last study dated 30/8/2025

I- CHEST: -

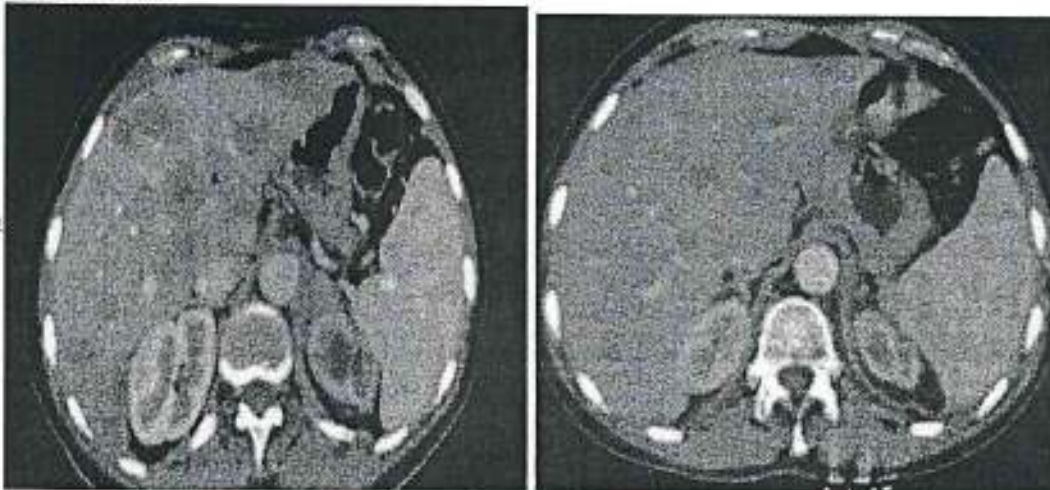
Findings:

- Evidence of left MRM , still noted no sizable soft tissue masses to account for residual/recurrent neoplasia.
- No sizable left axillary lymph nodes.
- Right breast upper outer quadrant soft tissue lesion is still noted with **mild increase in size** measuring about 1.6 x 1 cm compared to 11.5x8 mm for sonomammography correlation. -
- **Regression in size** of right axillary lymph nodes are noted, the largest seen measuring about compared to 8 x4 mm 16x7 mm.
- Both lungs show few upper lobar emphysematous changes as well as scattered atelectatic bands,
- No sizable nodules, masses or consolidative patches.
- No sizable hilar or mediastinal lymph nodes.
- Patent tracheo-bronchial tree.
- Normal CT appearance of the mediastinal vessels.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.

ABDOMEN AND PELVIS: -

Findings:

- The liver is enlarged showing marked **regression in size and number** of multiple scattered rather well defined variable sized focal lesions, showing peripheral enhancement and hypodense center, the largest seen at segment VI measuring about 15 x 12 mm compared to 29x27 mm.



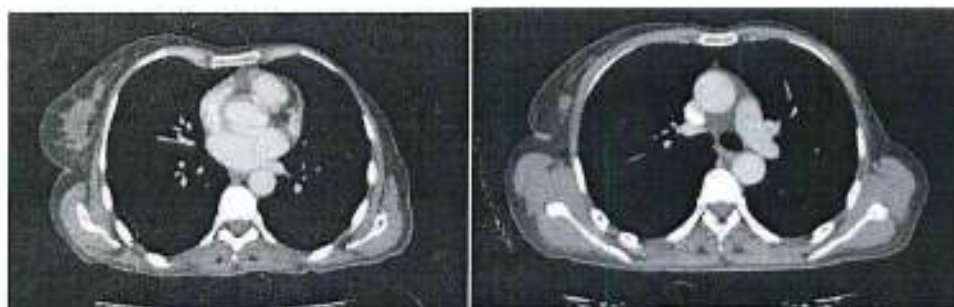
Patient Name: Souad Gamal Atteya Ibrahim	Date : 30/08/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 52 Year(s)
Medical ID: 2354/2025	Pat. ID: 111192

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

Clinical History:

- Known case of left breast cancer, no available previous films for comparison.

CT SCAN OF THE CHEST: -



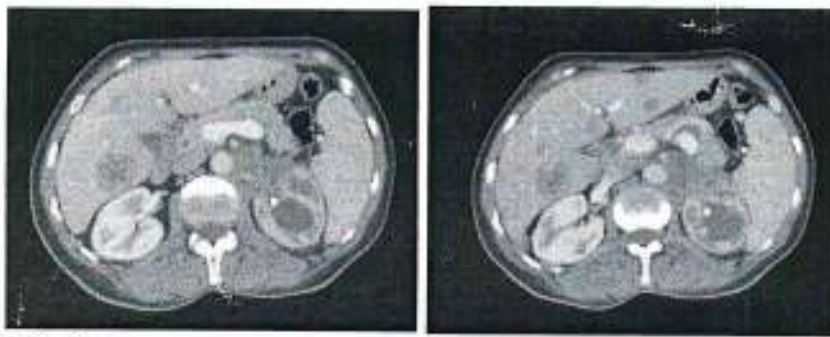
Findings:

- Evidence of left MRM with no sizable soft tissue masses to account for residual/recurrent neoplasia.
- No sizable left axillary lymph nodes.
- Right breast upper outer quadrant soft tissue lesion is noted measuring about 11.5x8 mm for sonomammography correlation.
- Few right axillary lymph nodes are noted, the largest seen measuring about 16x7 mm.
- Both lungs show few upper lobar emphysematous changes as well as scattered atelectatic bands,
- No sizable nodules, masses or consolidative patches.
- No sizable hilar or mediastinal lymph nodes.
- Patent tracheo-bronchial tree.
- Normal CT appearance of the mediastinal vessels.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.

OPINION:

- Evidence of left MRM with no sizable soft tissue masses to account for residual/recurrent neoplasia.
- No sizable left axillary lymph nodes.
- Right breast upper outer quadrant soft tissue lesion for sonomammography correlation.
- Few right axillary lymph nodes.

CT SCAN OF THE ABDOMEN AND PELVIS: -



Findings:

- The liver is enlarged (17 cm) showing multiple scattered rather well defined variable sized focal lesions, showing peripheral enhancement and hypodense center, the largest seen at segment VI measuring about 29x27 mm.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- The spleen is mildly enlarged (13.5 cm) with no focal lesions seen.
- The left adrenal gland is seen replaced by large ill defined heterogenous soft tissue mass lesion, measuring about +/- 4.8x4.1 cm seen abutting/inseparable from the left side of the abdominal aorta medially, abutting/inseparable from the tail of pancreas laterally and encasing/infiltrating the left renal vessels and left ureter with subsequent left moderate pelvialyceal system dilatation. DJ stent is noted.
- Normal size of both kidneys with no right sided stones or back-pressure changes.
- Normal appearance of the pancreas and right suprarenal gland.
- Average filling of the urinary bladder with no sizable stones or masses.

Opinion:

- *Multiple scattered hepatic focal lesions as fore-detailed, likely of metastatic nature.*
- *Left adrenal gland large ill defined heterogenous soft tissue mass lesion with extensions and effects as fore-detailed.*

Marina Essam, MSc
Dr.Noura Nabil,MD

Signature Noura Nabil
02/09/2025 3:07 AM

Patient's Name: souad gamal atteya ibrahim
Date: 2026-03-24
Referred by: Prof.Dr.
Hospital: Goustave Roussey International Egypt

**MULTISLICE POST CONTRAST CT SCAN OF THE
CHEST, ABDOMEN AND PELVIS:**

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported left breast carcinoma.

Compared to the last study dated 07-12-2025 ,the current study revealed .

(I) Chest:

- Left mastectomy with clear operative bed .
- Still noted Left lingula subpleural reticulations are noted.
- Bilateral lower lobar mosaic attenuation of small airway disease.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.
- Still noted right breast ROQ small lesion measures 14 x 10 mm compared to 16 x 10 mm at last study ,for sono mammographic correlation.

(II) Abdomen and pelvis:

Patient's Name: souad gamal atteya ibrahim
Date: 2026-03-24
Referred by: Prof.Dr.
Hospital: Goustave Roussey International Egypt

- The liver is enlarged in size showing homogenous CT density with bilobar hepatic hypodense focal lesions , the largest seen at segment VI measures 9 x 10 compared to 15 x 12 mm .
- Patent and homogeneously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, right kidneys, aorta and IVC.
- Left moderate hydrouretronephrosis with double J catheter in place .perinephric hypodense lesion is noted at renal hilum
- Bulky left suprarenal gland, no sizable masses are noted .
- Regular filling of the urinary bladder showing no stones, masses .
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- No worrisome lytic or sclerotic bony lesions .
- No vertebral structural collapse .

OPINION:

- Regressive course regarding hepatic focal lesions
- Rather stationary course rest of the study

Dr.Reham Khedr, MD

Dr.Rahma Khader

Dr.Rahma Khader

ECHO REPORT

NAME : *سعاد جمال عيسى*

DATE: *4 / 3 / 2026*

ID NUMBER : */*

SEX : *F* AGE: *54*

EF	<i>64%</i>	55-75%	PWT	<i>0.8</i>	0.6-1.1cm
LVEDD	<i>4.6</i>	3.5-5.8cm	AO.R	<i>2.3</i>	3-3.7cm
LVESD	<i>2.3</i>	2-4cm	LA	<i>3.7</i>	2-4cm
FS	<i>30%</i>	25-45%	RV		0.7-2.3cm(supine)
SWT	<i>0.9</i>	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions *Normal*

LV Wall Thickness *Normal*

LV Diastolic Function *GI*

SWMA *No RWMA*

LA and Ao Root Dimensions *Normal*

Mitral Valve *N*

Aortic Valve *N*

Tricuspid Valve *N*

Pulmonary Valve

Right Side and Systolic Function *TAPSE = 2.5*

Intracardiac Masses and Effusion

DR. *طه الكونى*